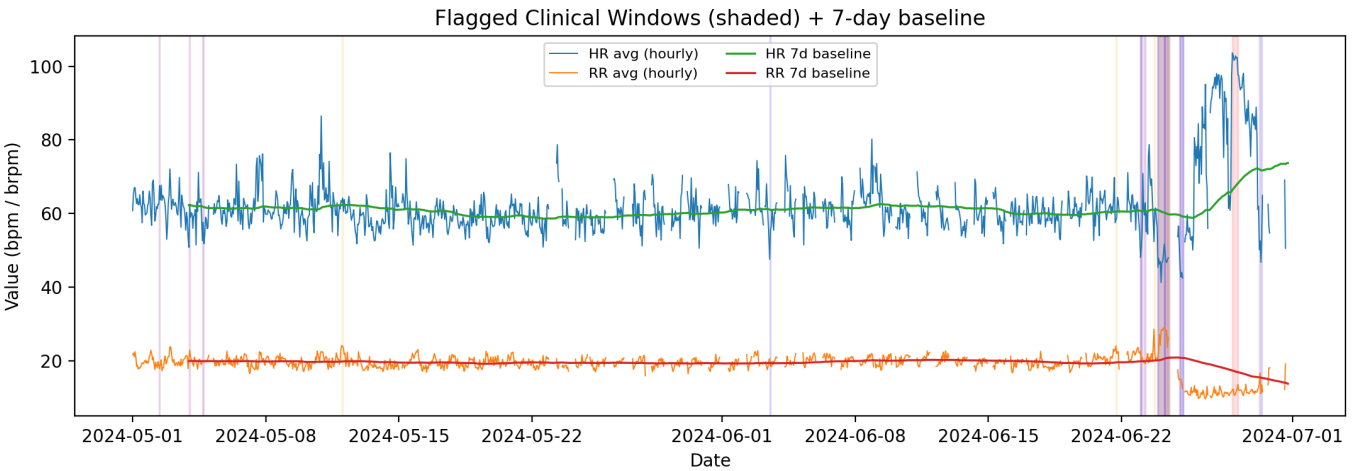


B) Physician/Cardiology Summary — Vital Trend Intelligence

Data window: 2024-05-01 → 2024-06-30 | Hourly

Metric	Mean	P95	Min	Max
HR avg (bpm)	62.1	80.3	41.3	103.6
RR avg (brpm)	19.1	22.3	9.7	29.1



Top episodes (by duration)

Flag	Start	End	Hours
Tachypnea (RR avg >24)	2024-06-24 00:00	2024-06-24 09:00	10
Severe bradycardia (HR min <45)	2024-06-23 21:00	2024-06-24 05:00	9
Bradycardia (HR avg <50)	2024-06-23 22:00	2024-06-24 05:00	8
Tachycardia (HR avg >100)	2024-06-27 20:00	2024-06-28 02:00	7
Severe bradycardia (HR min <45)	2024-06-25 01:00	2024-06-25 05:00	5
Bradycardia (HR avg <50)	2024-06-24 07:00	2024-06-24 11:00	5
Bradycardia (HR avg <50)	2024-06-25 02:00	2024-06-25 05:00	4
Severe bradycardia (HR min <45)	2024-06-24 07:00	2024-06-24 10:00	4
Severe bradycardia (HR min <45)	2024-05-04 16:00	2024-05-04 17:00	2
Severe bradycardia (HR min <45)	2024-06-23 00:00	2024-06-23 01:00	2
Severe bradycardia (HR min <45)	2024-06-29 06:00	2024-06-29 06:00	1
Tachypnea (RR avg >24)	2024-06-24 11:00	2024-06-24 11:00	1

Interpretation prompts (needs clinical context):

- **Bradycardia:** sleep physiology vs sinus node dysfunction/AV block vs medication effect (β -blocker, non-DHP CCB, digoxin, amiodarone); consider ECG/telemetry review if symptomatic/new.
- **Tachycardia:** pain, infection/sepsis, dehydration, anemia, hypoxia, volume overload, anxiety, atrial arrhythmia; correlate with rhythm/hemodynamics and symptoms.
- **Tachypnea:** correlate with SpO₂, work of breathing, CHF/pulmonary process; co-occurrence with HR rise increases concern for physiologic stress/decompensation.
- **Data integrity:** low sample-count hours and gaps can exaggerate extremes; hourly averages are typically more robust than max values.